CENTRAL GERIATRIC CLINIC - CONSULTATION SUMMARY

Patient Name: Ananth Narayan | Age: 74 | Sex: Male | UHID: CCG-GER-2026-1006
Attending Geriatrician: Dr. Malini Sengupta, MD

CHIEF COMPLAINTS:

Recurrent morning occipital headaches, postural dizziness upon standing up from bed, and ambulatory BP fluctuations.

PRIMARY DIAGNOSIS:

1. Stage 2 Essential Hypertension with Postural Orthostatic Hypotension.
2. Benign Prostatic Hyperplasia (BPH).
3. Age-Related Mild Cerebral Small Vessel Ischemic Disease.

CLINICAL INVESTIGATIONS & LAB RESULTS:

- Blood Pressure Sitting: 172/94 mmHg -> Standing (3 min): 138/82 mmHg (34 mmHg systolic drop)
- Heart Rate: 64 bpm | Serum Creatinine: 1.1 mg/dL | eGFR: 68 mL/min
- Serum Sodium: 140 mEq/L | Serum Potassium: 4.4 mEq/L | Timed Up-and-Go (TUG): 12.8s.

DISCHARGE MEDICATIONS & PRESCRIPTION:

1. Tab. Amlodipine 5 mg - 1 tablet orally once daily morning after breakfast.
2. Tab. Telmisartan 40 mg - 1 tablet orally once daily morning after breakfast.
3. Tab. Silodosin 8 mg - 1 tablet orally once daily at bedtime after dinner.
4. Sachet Cholecalciferol 60,000 IU - once weekly with milk for 8 weeks.

CONTRAINDICATIONS & MEDICATION ALERTS:

- FALL RISK ALERT: Silodosin + Antihypertensives cause significant postural hypotension. Sit at bedside for 60 seconds before st
- Avoid nasal/oral decongestants (Pseudoephedrine) which spike BP and cause acute urinary retention.

DIETARY & LIFESTYLE GUIDANCE:

- Maintain daily sitting and standing BP log.
- Install grab bars and nightlights in bathroom corridor to prevent nocturnal falls.